



PROTOCOL

CAMHS Crisis Connect Entry Protocol

Scope (Staff):	All staff
Scope (Area):	CAMHS Crisis Connect

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This protocol is currently under review to be incorporated into the [CAMHS Crisis Connect Model of Care](#). The content remains current throughout this review process.

Aim

Describes the function, role and responsibility of the CAMHS Crisis Connect service provided by CAMHS at Perth Children's Hospital (PCH).

Risk

Failure to adhere to this protocol may contribute to a delay in the provision of relevant mental health treatment and care, which can result in longer periods of mental health treatment and care.

Background

The CAMHS Crisis Connect service provides specialist mental health consultation, liaison, crisis management and assessment 24 hours per day, 7 days per week.

There are five components to the service model are:

- Specialist tele-mental health urgent support for young people, parents and carers in crisis, including carers of children and young people in 'out of home' care:
 - This component of the service provides brief specialist child and adolescent telephone mental health assessments and crisis intervention via telephone.
- Specialist emergency tele-mental health assessments for young people calling and identified as requiring a mental health assessment, as an alternative to presenting to PCH. This may include locations such as home environments,

schools, general practitioners (GP), Tier 2 services or metropolitan emergency departments (ED).

- Specialist emergency consultation, advice and guidance with other professionals in metropolitan EDs and in the community; including GPs, school psychologists or school nurses.
- CAMHS Crisis Connect will be available to complete specialist emergency face-to-face mental health assessments for young people who present to PCH ED.
- CAMHS Crisis Connect will provide one 24-hour tele-mental health follow-up of young people who have utilised the service (daily during non-business hours).

Principles

- [The Charter of Mental Health Care Principles MHA 2014](#) guides service delivery for children and young people accessing services with mental health presentations.
- CAMHS Crisis Connect assessments should be provided to a young person with a safe adult present. Every effort must be made to ensure the availability of a safe adult. If this is not possible crisis support or direction to an ED for a face-to-face assessment may be provided.

Inclusion Criteria

- Young people up to the age of 18 years old requiring urgent mental health support or emergency tele-mental health assessment while in a community setting e.g. school, non-government organisation.
- Young people up to the age of 16 years old presenting to PCH ED or other metropolitan EDs with mental health concerns.
 - As per [PCH Admission and Age of Eligibility](#) policy.
- Families, carers and professionals seeking mental health support and guidance about a young person up to the age of 18. This includes professionals such as GPs, school psychologists, school nurses and clinicians in other ED's.

Exclusion Criteria

- Young people calling the 1800 line who are over 18 are to be redirected to MHERL.
 - MHERL phone numbers:
 - Metro callers 1300 555 788 (local call)
 - Peel callers 1800 676 822 (free call)
 - Rural Link 1800 552 002.

Caller Identified as at Imminent and Significant Risk

When a young person, family or carer, who has been deemed on assessment to be at imminent and significant risk to themselves and/or others or report high risk (e.g. due to environmental triggers, unsafe adults present etc) the young person, family or carers will be advised to attend ED and/or follow their crisis plan.

Triage

All phone calls received by the 1800 number, will be triaged as per the following process.

Young Person and Family - Triage Assessment

- CAMHS Crisis Connect clinicians provide a welcome, brief description of the service and brief triage assessment.
 - See CAMHS Crisis Connect Distribution of Safety Information and Digital Welcome Pack Work Instruction.
 - See [CAMHS Crisis Connect Survey Work Instruction](#).
- PSOLIS background check completed.
- PSOLIS triage/service event entered.

Outcomes of triage call may include:

- Safety planning, information about community supports, referral to Community CAMHS/other community services;
 - Where a referral to Community CAMHS is made, the CAMHS Crisis Connect clinician is to handover to Community CAMHS.
 - Also see [CAMHS Clinical Handover Guideline](#) and SSCD Care of Transfer of Summary form ([printable](#) and [writable](#)).
- An emergency tele-mental health assessment being offered; or
- Recommendation to present to an ED.

Community Assessment Safety Screen (CASS; MR506.00)

If the young person is located in a community setting and a mental health assessment is deemed necessary, the Community Safety Assessment Checklist (CASS) will be used to assess if it is safe to progress to a tele-mental health assessment of a young person in their home setting without the face-to-face presence of a health professional.

Please refer to the [CAMHS Consent Procedure](#).

Mental Health Assessment via Telehealth with Young Person and Family/Professional in the Community

Young person may be located at home, in a Metropolitan ED or in a community setting e.g. school, NGO.

CAMHS Crisis Connect clinician:

- Completes background search in PSOLIS.
- Undertakes initial assessment and risk assessment as per [CAMHS Clinical Assessment Policy](#) and [CAMHS Risk Assessment and Management Policy](#).
- In consultation with the CAMHS consultant psychiatrist, develops a plan in collaboration with the child and family.
 - The management plan must be endorsed by the CAMHS Crisis Connect psychiatrist during working hours and on call psychiatrist after hours.
- Ensures understanding by young person, family and/or professional of suggested steps/management plan.

Where Young Person is in a Metropolitan ED (non-PCH)

- CAMHS Crisis Connect clinician discusses plan with mental health team in ED and then with young person and parent.
- Once there is agreement on a plan, it is finalised and communicated to the Metropolitan ED mental health team.
- The Metropolitan ED mental health team will then communicate the plan to the family.

Outcome of Assessment

Community CAMHS/Community Organisation Referral

- CAMHS Crisis Connect clinician develops draft management plan and ensures young person and family and/or professional understanding of suggested plan and agree to be referred to the community service.
- Community CAMHS referral:
 - If not currently active with Community CAMHS, the young person and family are provided with Community CAMHS information and welcome pack.
 - CAMHS Crisis Connect clinician sends copies of the CAMHS Initial Assessment SSCD form and CAMHS Risk Assessment and Management Plan (RAMP) and SSCD Care Transfer Summary to Community CAMHS team. This is sent to the Choice Co-ordinator (and Case Manager for active Community CAMHS clients) with the generic clinic email copied in.
 - The CAMHS Crisis Connect clinician provides verbal handover to Community CAMHS (Case Manager or Choice Co-ordinator as appropriate).
 - Community CAMHS should contact the family within 2 working days to arrange an appointment.

- Also see [CAMHS Clinical Handover Guideline](#) and SSCD Care of Transfer of Summary form ([printable](#) and [writable](#)).

Admission

- If an admission to the CAMHS Ward 5A is necessary, this will be organised directly.
- The psychiatric registrar and/or STARS resident medical officer will complete the physical examination on the CAMHS Ward 5A, as per [CAMHS IPU Entry Protocol](#).
- Patient will not be admitted through the PCH ED.
- If transport is required, the least restrictive option must be used.
- Where admission to CAMHS Ward 5A is delayed, CAMHS Crisis Connect in collaboration with ED doctors, will provide tele-mental health review.
- CAMHS Crisis Connect clinician must consult with CAMHS Crisis Connect consultant psychiatrist during business hours and the on-call registrar/consultant psychiatrist outside of business hours.
 - If a bed on CAMHS Ward 5A is unavailable, consider using general ward bed. Refer to [PCH Admission and Care of Mental Health Patients in General Wards Procedure](#).
- Also see [CAMHS Clinical Handover Guideline](#) and SSCD Care of Transfer of Summary form ([printable](#) and [writable](#)).

Mental Health Assessment of young people presenting to PCH ED

Patients presenting to PCH ED will be assessed by the CAMHS Crisis Connect CNS or the psychiatry registrar in liaison with the on-call psychiatrist.

ED Triage Nurse Completes Triage and Determines Category

- Where a mental health assessment is indicated the PCH ED triage nurse informs the CAMHS Crisis Connect clinician and conveys young person's name, presentation and determined category.
- ED triage nurse provides patient and carer with the "What to Expect in PCH ED" leaflet.
- Where the mental health assessment indicates admission is required and a mental health inpatient bed is not available on CAMHS Ward 5A, admission to a medical bed is considered.
- See [PCH Admission and Care of Mental Health Patients in General Wards Procedure](#).

Actions Where Young Person is Discharged:

- CAMHS Crisis Connect clinician gives young person and family the SSCD Care Transfer Summary.
- CAMHS Crisis Connect clinician sends a copy of SSCD Care Transfer Summary to the referrer/other stakeholders.
- Refer to the [CAMHS Clinical Handover Guideline](#).

Medical Assessment by ED Doctor:

- All patients must be seen by an ED doctor and be medically cleared before leaving ED.
- Timing of medical assessment can be joint, pre or post the CAMHS Crisis Connect assessment.
- The outcome of the medical assessment must be recorded in the progress notes.

Information for Young People, Families and Stakeholders

Consumer information on CAMHS Crisis Connect is available online via the:

- [CAMHS website](#); and
- [CAMHS Health Point hub page](#).

Related CAHS internal policies, procedures and guidelines
Admission to CAMHS Ward 5A Procedure
CAMHS Access Policy
CAMHS Clinical Assessment Policy
CAMHS IPU Entry Protocol
CAMHS Risk Assessment and Management Policy
CAMHS Clinical Handover Guideline
PCH Admission and Care of Mental Health Patients in General Wards Procedure
PCH Admission and Age of Eligibility
State-wide Standardised Clinical Documentation (SSCD) for Mental Health Services
CAMHS Crisis Connect Survey Work Instruction

CAMHS Crisis Connect Distribution of Safety Information and Digital Welcome Pack Work Instruction.
References and related external legislation, policies, and guidelines (if required)
The Charter of Mental Health Care Principles MHA 2014
Useful resources (including related forms)
List and hyperlink the titles of useful resources, do not hyperlink MR forms
Community Assessment Safety Screen (CASS) MR506.00. Available only in printed format, order from Print Media Group.
SSCD CAMHS RAMP Printable Form
SSCD CAMHS RAMP Writable Form
CAMHS SSCD Initial Assessment Printable form
CAMHS SSCD Initial Assessment Writable form
SSCD Care Transfer Summary Printable form
SSCD Care Transfer Summary Writable form

This document can be made available in alternative formats on request.

Document Owner:	Head of Department and Director PCH CAMHS		
Reviewer / Team:	CAMHS ETS Team		
Date First Issued:	25/08/2021	Last Reviewed:	N/A
Amendment Dates:	6/09/2022	Next Review Date:	06/06/2026*
Approved by:	Head of Department and Director PCH CAMHS	Date:	22/04/2021
Endorsed by:	CAMHS Safe Systems and Practice Oversight Committee	Date:	25/08/2021
Standards Applicable:	NSQHS Standards:  NSMHS: 2, 3, 6, 7, 8, 9, 10.2, 10.3, 10.4 Child Safe Standards: 2, 3, 5, 7		
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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

*Note: CAMHS Director Clinical Services approved a 3-month extension of the review date to 06/06/2026.